

DoktorABC Aesthetics MVP Launch Plan

Expand into Botox treatments using a telemedicine-driven model (online consult & in-clinic injection) to make aesthetic care more accessible, safe, and convenient.

For: Director of Product | DoktorABC
Last Edited: 05.11.25
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Company Context & Why Now



DoktorABC Today

Proven telemedicine platform across UK, Germany, France, Switzerland

- 35+ treatment categories, 2,500+ medications
- 500+ partner pharmacies
- 15,000+ reviews (4.7★ on Trustpilot)



Why Aesthetics, Why Now

Botox market ~\$5.3B in 2025, to ~\$10B by 2032 (~10% CAGR)

- Rising demand for preventative treatments
- Digital gap in aesthetic clinic experiences
- Strategic fit with our telemedicine expertise

Mission: **make qualified medical care and treatments accessible, safe, and convenient, end-to-end**

This new vertical directly extends DoktorABC's core mission by applying our proven telemedicine model to a growing market, ensuring patients receive high-quality, transparent, and convenient aesthetic care.

Market Data Supporting Growth Projections

Our entry into the aesthetics market is bolstered by robust market expansion, shifting demographics, and a clear digital service gap that DoktorABC is uniquely positioned to fill.

€11.5B

Projected Market Size (2025)

Germany Medical Aesthetics market at its highest projection.

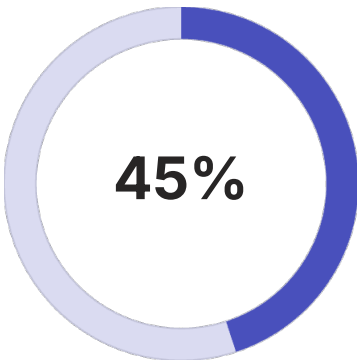
17.6%

YoY Growth & Long-term CAGR

Highest estimated year-over-year growth and Compound Annual Growth Rate through 2030-2035.

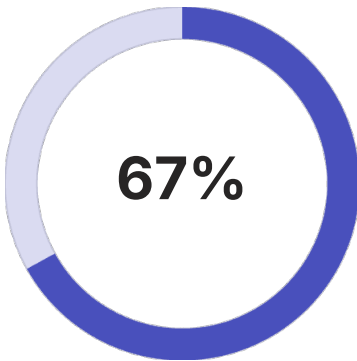
The market demonstrates significant growth potential, with estimates for the German medical aesthetics sector reaching up to €11.5B by 2025, driven by a long-term CAGR of up to 17.6%.

Key Demographics Driving Growth



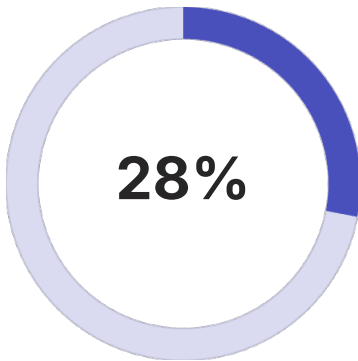
Ages 25-34 Uptake

Increase in treatment uptake for this age group from 2022-2024.



"Preventative Botox" Trend

New patients under 35 adopting preventative treatments.



Male Demographic Growth

Significant growth in male patient numbers in the aesthetic market.

Younger demographics and male patients are increasingly seeking aesthetic treatments, particularly "preventative Botox," indicating a broadening market appeal.

Digital Opportunity Gap



No Online Booking

Traditional clinics still lack basic digital convenience.



Telemedicine Integration

Low adoption of telemedicine among aesthetic providers.



No Price Transparency

Clinics often fail to publish pricing, hindering patient decision-making.

A significant digital gap exists, with a majority of traditional clinics failing to offer online booking or price transparency, and very few integrating telemedicine. This presents a prime opportunity for DoktorABC to lead with a digitally-forward approach.

The Problem & Mission Fit

Patient Experience Gap

Many hesitate due to fear of unnatural outcomes, lack of trust, inconvenience, and intimidation especially first-timers.

Fragmented Journey

Online research disconnected from in-person injection. Clinics operate offline with little tech, causing inefficiencies and lost opportunities.

Mission Alignment

Extends DoktorABC's mission of "**discreet, fast, qualified care**" by uniting digital consultation and physical treatment into one seamless service.

Company mission:

Provide **fast, discreet, and safe access** to licensed medical care, from online doctor consultation to EU-certified pharmacy fulfillment

Vertical mission:

Make **medically supervised aesthetic injectables accessible and transparent**, offering digital consults and upfront pricing, vetted in-person treatment, and guided follow-up.



Segment Focus – Target Persona



First-timers: "Decide Safely & Book Fast"

Market Size: [24% of current users](#) (ages 19-34) + [fence-sitters](#)

Demographics: [Women](#) 25-40, 94% patients, urban, €50K+ income

Main Objection: Fear (safety, naturalness, "will I look fake?")

JTBD: Make their first aesthetic procedure decision with confidence and minimal friction

Key Behaviors:

- Research 3.2 hours across devices (83% mobile)
- 80%+ read reviews before booking
- [10x](#) more likely to convert if responded to within 5 minutes
- Preventative mindset driven by social media/Zoom



Switchers: "Same Results, Less Effort" (PRIORITY SEGMENT)

Market Size: 30-40% of existing injectable users

Demographics: [Women](#) 35-54 (57% of all Botox patients), experienced with procedures

Main Objection: Switching cost (relationship with current provider, consistency concerns)

JTBD: Maintain aesthetic results while reducing time/effort and gaining price transparency

Key Behaviors:

- [Quick decision-makers](#) (compare 2-3 options, then decide)
- High price sensitivity-know market rates
- [Value online](#) booking over phone tag
- [Dissatisfaction](#) with current clinic (wait times, scheduling friction)
- [72%](#) book with whoever responds first



Upgraders: "Learn & Try Once"

Market Size: 15-20% fence-sitters intend to try injectables within 12 months

Demographics: [Women](#) 30-45, high skincare spend (€100+/month), beauty-conscious

Main Objection: Low urgency ("my skincare is working fine for now")

JTBD: Transition from topical skincare to injectables with minimal commitment/risk

Key Behaviors:

- [Longest research cycle](#) (weeks to months, multiple touchpoints)
- Comparing injectable cost/ROI to premium skincare
- Want "trial" or starter packages (lower commitment)
- Influenced by skincare influencers + friends' results
- Concerned about permanence (need reversibility messaging)

Segments: Economic Comparison & Prioritization

Key Assumptions for LTV:CAC Calculations (18-month horizon)

Avg Treatment Price (P): €400 | Take Rate (t): 30% | Variable Platform Cost (v): €15/treatment | Contribution per treatment (C): €105 (P × t - v = €400 × 0.30 - €15 = €105)

Expected completed treatments per acquired paying patient: • S1 First-timers = 2.5 • S2 Switchers = 4.5 • S3 Upgraders = 2.0

	First-Timers	Switchers	Upgraders
Market Share	24% + growing	30-40%	15-20%
JTBD	Decide safely & book fast	Same results, less effort	Learn & try once
Main Objection	Fear/Trust	Switching cost	Low urgency
LTV:CAC	1.05-1.31:1 €263 LTV ÷ €250 CAC = 1.05:1	3.15-3.94:1 ⭐ €473 LTV ÷ €150 CAC = 3.15:1	0.95-1.17:1 €210 LTV ÷ €220 CAC = 0.95:1
Repeat Rate	40-45%	60-70%	45-50%
Phase 1 Priority	Secondary	PRIMARY	Deferred

Prioritization Logic

Rule Applied: "Only keep segments that change funnel, pricing, or operations"

All 3 segments qualify

- 1

S2 (Switchers): PRIMARY

Best economics (3.15-3.94:1 LTV:CAC), fastest conversion.
- 2

S1 (First-timers): SECONDARY

Volume opportunity, crucial for testing education content.
- 3

S3 (Upgraders): DEFERRED

Requires longer nurture and content library build-out.

Blended Target Metrics

€383

Overall LTV

Average customer lifetime value.

2.27:1

Target LTV:CAC

Path to 3:1 benchmark.

€169

Overall CAC

Average customer acquisition cost.

≈52%

Overall Repeat Rate

Expected customer retention at 6 months.

* Scenario B: Avg price €500 lifts blended LTV:CAC to 2.92:1 for the same mix

Target Persona: Diana (Switcher)

Diana, 42 | €60K-€150K+ | Berlin/Munich/Hamburg/Frankfurt | 3+ years Botox user, frustrated with current clinics

“I’ve been getting Botox for 3 years and my clinic frustrates me. I want the same results with way less hassle.”

Pain Points & Current Experience

01	02	03	04
Scheduling Friction	Price Opacity	Consistency Fear	Time Waste
48hr callbacks, no online booking → Feels disrespected	Unclear quotes, hidden fees → Suspicious of overcharging	Different practitioners, varying results → Anxious about quality	45min total for 15min procedure → Resentful of inefficiency

05
No Follow-up
Forgets rebooking, no reminders → Annoyed at self-management

Primary JTBD: same results WITHOUT the scheduling headache.

Secondary JTBD: Know she’s not overpaying

Emotional JTBD: Feel efficient, respected, and in control.

The Main Objection to Switching - The Switching Cost Barrier

Switching feels like breaking up. What if the new place is worse? New practitioner, new technique? Could waste money and feel disappointed.

Value Proposition

Same results, less effort, clear price

Instant Clinic Comparison & Real-Time Slots

Compare clinics instantly and view real-time availability, leading to a **60-second booking process**.

Transparent Pricing

Clear pricing by area and units means **no hidden fees or surprises**.

Switching Bonus & Saved Preferences

Benefit from a switching bonus and saved preferences, ensuring **fast and easy repeat bookings**.

Why Switch to DoktorABC? A Comparison

Pillar	Current Clinic	DoktorABC	Impact
Convenience	Phone → wait 48hrs → negotiate → inflexible	Book 90 sec → see options → pick time → done	72% book with whoever responds first
Transparency	Vague pricing → different quotes → suspicion	€350 dose + €75 doctor + €25 clinic = €450 visible	Removes price-matching objection
Consistency	Different doctors → variable results → anxiety	Same doctor → same technique → guaranteed repeats	Removes biggest switching barrier
Automation	Diana remembers → calls → negotiates → books	SMS month 2.5 → one-click rebook pre-assigned	60-70% repeat rate vs 40%
Risk Reversal	If she switches & regrets, stuck & wasted	€50 credit + free touch-up if unsatisfied	Permission to switch safely

North Star Metric & Success Metrics

500

NSM Target

Paid Aesthetic Treatment Sessions Completed per Month - The count of completed, paid treatment events.

This captures both acquisition and retention, growing as we acquire new patients and bring them back for repeat sessions.

60%

Conversion Rate

Consultation → Treatment conversion.
Measures funnel efficiency from consult to procedure.

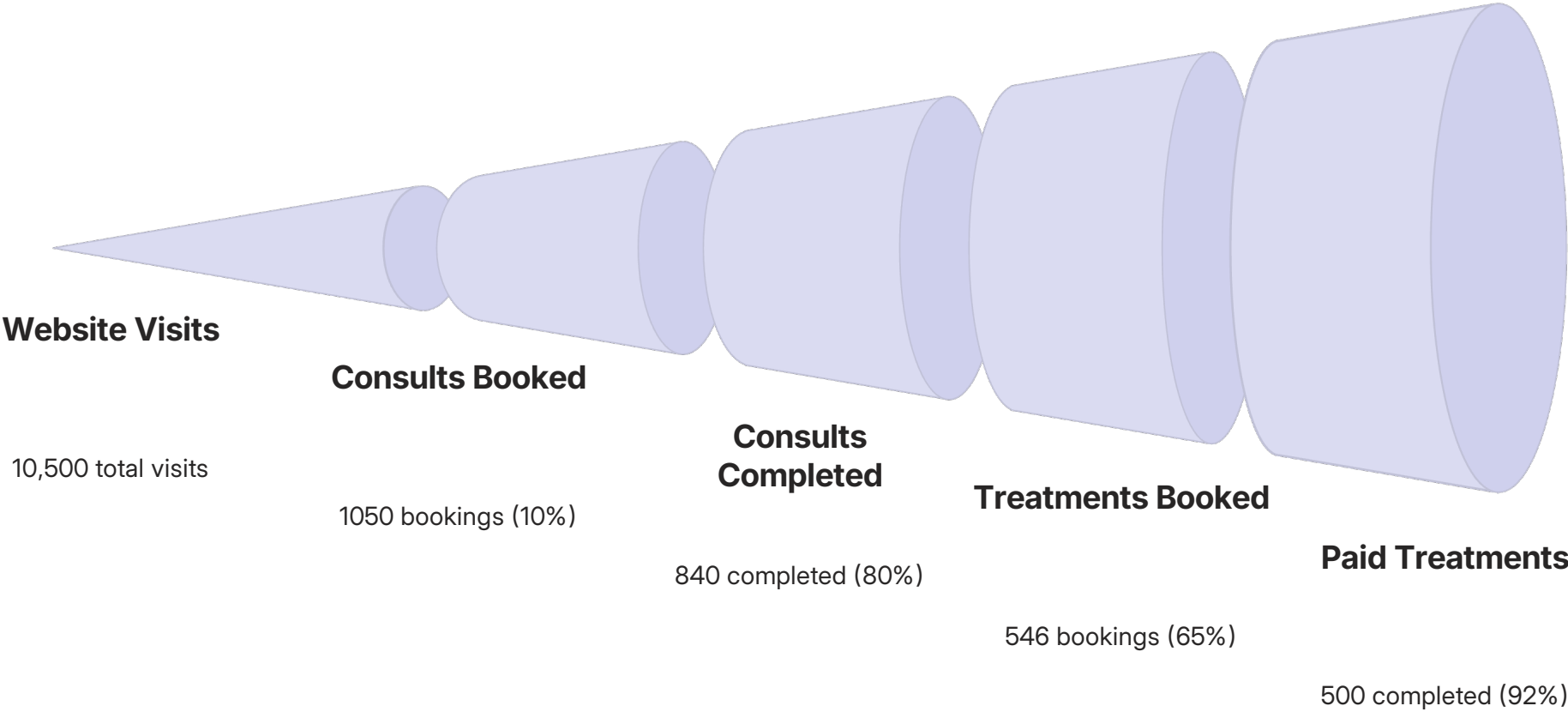
50%

Repeat Rate

Patients returning within 180 days.
Captures retention and loyalty-critical for sustainable growth.

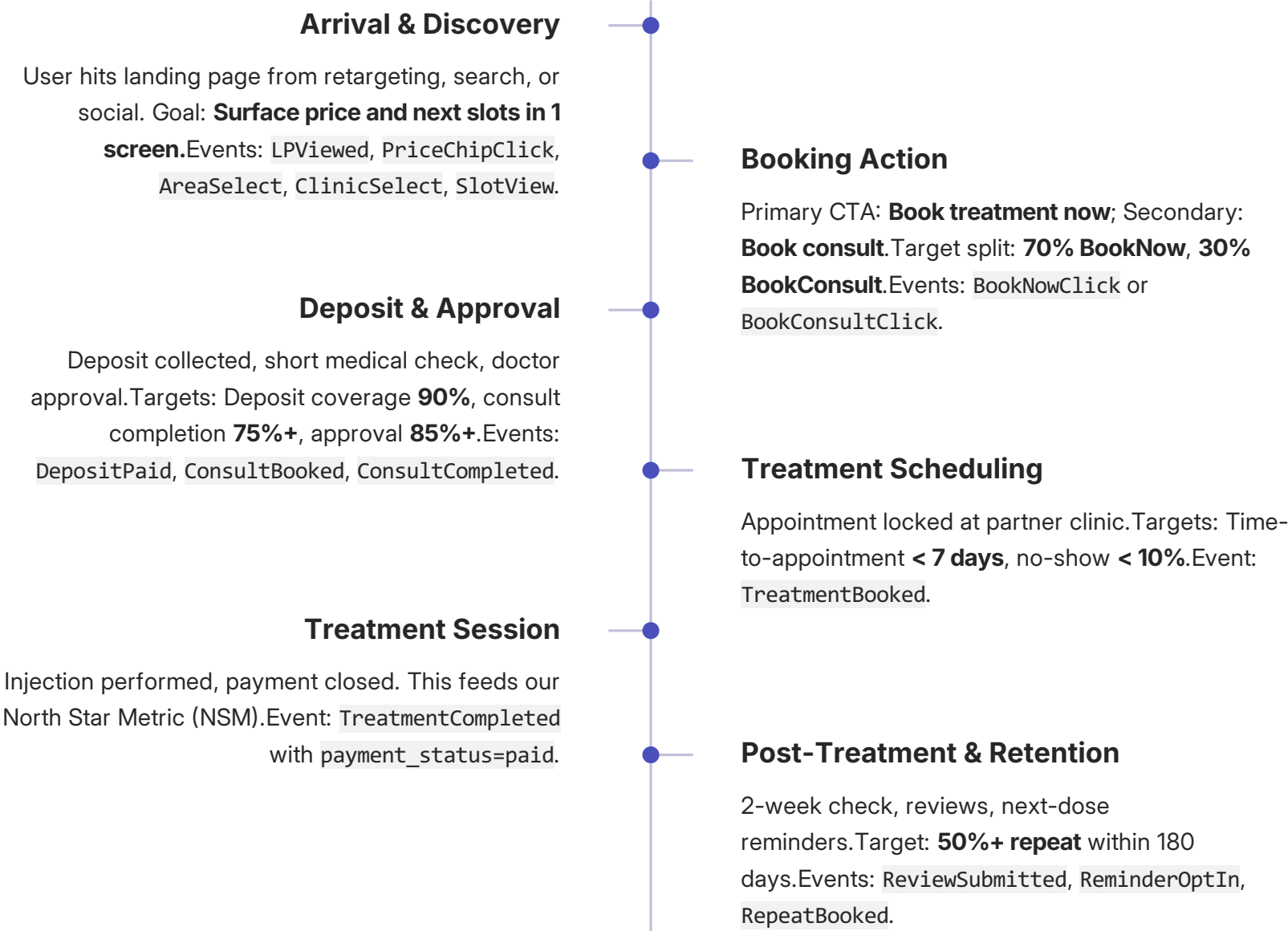
Guardrails -

- NPS ≥ 35 • Refund rate ≤ 3%
- No-show ≤ 10% • Adverse events = 0 reportable

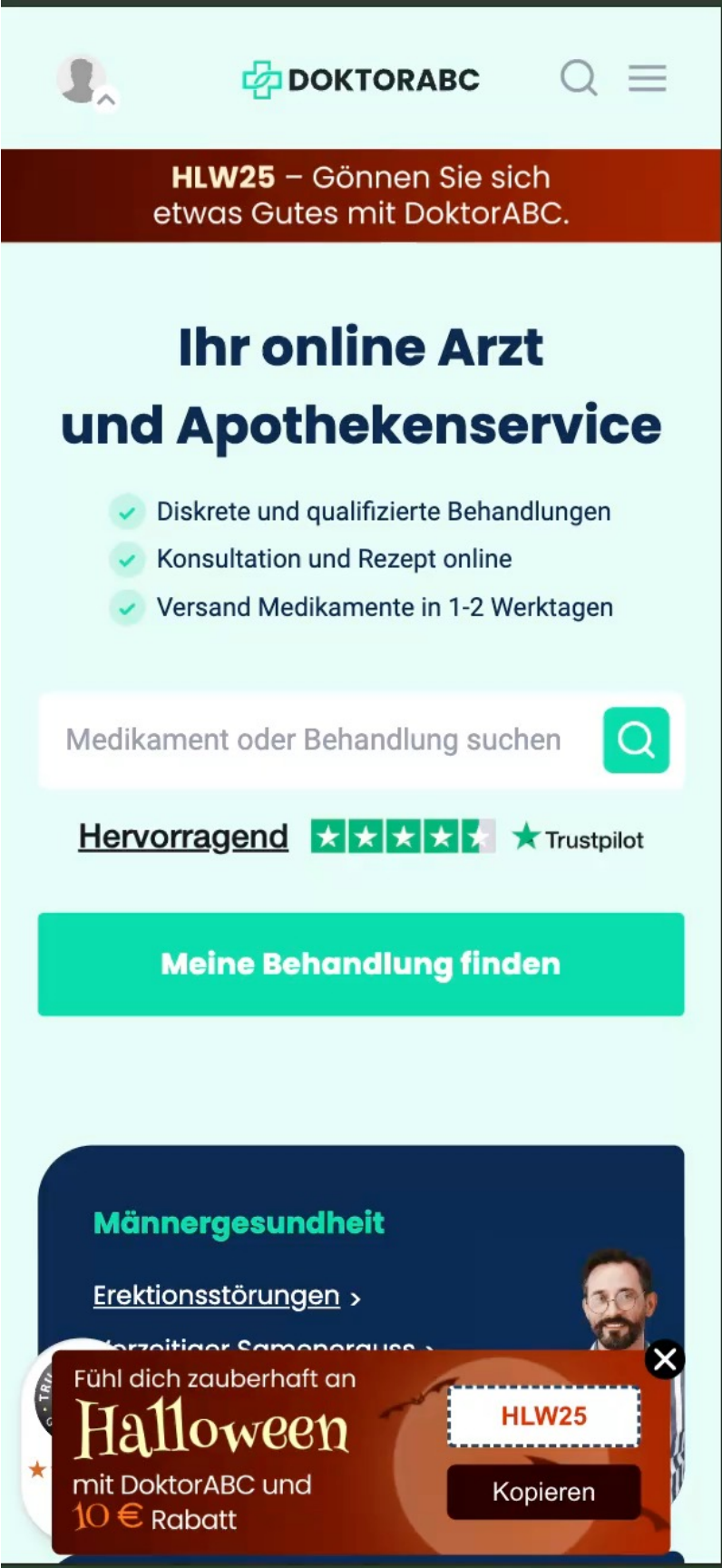


Pilot CAC assumption: S2 €135

End-to-End Funnel – S2 Switchers (Priority)



See price & slots -> Book now or consult -> Deposit paid & consult approved -> Treatment completed (paid) -> Review & repeat



[Mock](#)

[Repo](#)

Landing Page Strategy - S2 Switchers (Priority Segment)

Immediate Value & Action

Price Chip & Availability

"Botox from €350" + area selector updates estimate. Next 7 days availability grid for nearest clinic.

Price line: "From €350 per area. Typical full face €480-€650. No hidden fees."

Primary & Secondary CTAs

- Primary CTA: **Book treatment now**
- Secondary CTA: **Book consult only**

CTA line: "Book treatment now - 3 minutes"

Deposit Microcopy

"Hold your slot with a €50 refundable deposit."

Deposit line: "€50 deposit, deducted at checkout, fully refundable up to 24 hours before"

Guiding the Switcher

1. Fast Booking Block

One-click booking or direct from LP with deposit to cut no-shows. (Maps to H2 Booking Friction)

2. Transparent Pricing

Per-area price range, unit estimate, what's included, no hidden fees. (Maps to H4 Price Opacity)

3. Clinic & Practitioner Selector

Distance, earliest time, rating, skill badges, "same injector on repeat." (Maps to H1 Trust)

Trust cues: "EU-licensed injectors. 4.7 average from 15,500 reviews. Same injector on repeat."

4. Switching Incentive

"€50 switching credit" + quick import of prior plan. Reduces perceived switching cost.

5. Proof of Consistency

6 before-after pairs with identical lighting, technique notes, complication policy. Reinforces outcome certainty.

6. Light FAQ

5 answers max: dose, duration, downtime, touch-ups, safety contacts. (Minimal education to avoid S1 bias)

7. Retention Teaser

"Set reminders for 3-5 months" checkbox on confirmation page. (Maps to H3 Prompts)

A/B Testing & Success Metrics

A/B Cohorts

- A: Fast-track:** price chip + availability grid + deposit + one-click booking
- B: Follow-up:** no price chip + lead form + email scheduling

Key Metrics

Primary: Consult→TreatmentBooked uplift

Guardrails: NPS, refund rate, complaint rate

MVP Scope & Partner Model

MVP Scope (First 6 Months)

- **Geography:** 1-2 major cities (Berlin, Munich/London)
- **Services:** Core Botox areas (forehead, frown lines, crow's feet)
- **Scale:** 5-10 select aesthetic clinic partners
- **Goal:** ~500 completed treatments

Operational Model

Partner with existing licensed aesthetic clinics rather than owning facilities. DoktorABC manages digital platform; partners perform injections.

Benefits: Fast scaling, low CapEx, leverage experienced providers

Revenue Flow Example

Typical €400 Botox treatment: Platform retains ~30% (€120), Partner clinic receives ~70% (€280). Consult fee (€50) waived if patient proceeds with treatment, aligning everyone's interests.

Responsibilities

Platform:

Digital funnel and payments, verification of practitioners, data layer, reminders, support. 60 minutes, refund decision within 24 hours, support first reply under 5 minutes during business hours

Compliance: informed consent, medical questionnaire, audit trail, GDPR.

Clinic:

Perform injection, store product, clinical QA, adverse event reporting.
SLAs: no-show under 10 percent, complaint rate under 2 percent, rating 4.6 plus.

Quality controls:

Standard protocol sheet, random shadow checks on 10 percent of sessions, removal path for NPS below 3.

Unit Economics & Repeat Engine

€105

Contribution per Treatment

Price €400, take 30%, variable €15. $P \times t - v$.

Null

CAC by Segment

Switchers €120–€150
First-timers €200–€250
Upgraders €180–€220

1.1–1.4

Payback in Treatments

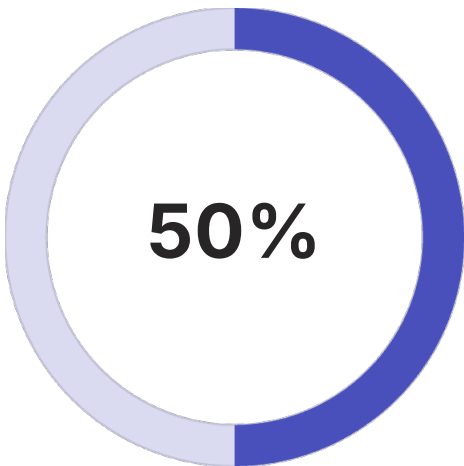
S2 only. $CAC \div €105$ Payback = $CAC \div$ contribution.

~286

Breakeven Volume

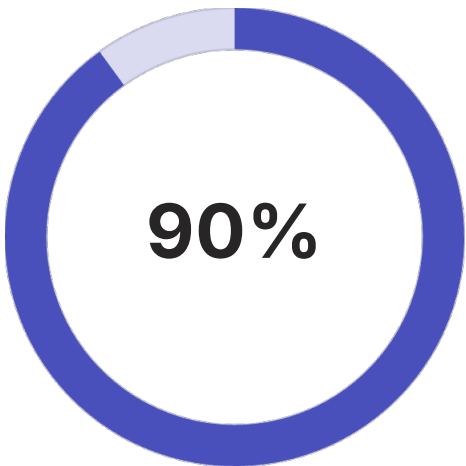
Fixed costs $\div$ €105 $\div$ €30k fixed costs needs
Incl. CAC: 465–800 by new patient mix

The Power of Repeat Business



Repeat within 180 days

Target for patients booking a repeat treatment within 180 days.



Treatment Completion

High completion rates ensure revenue realization and customer satisfaction.

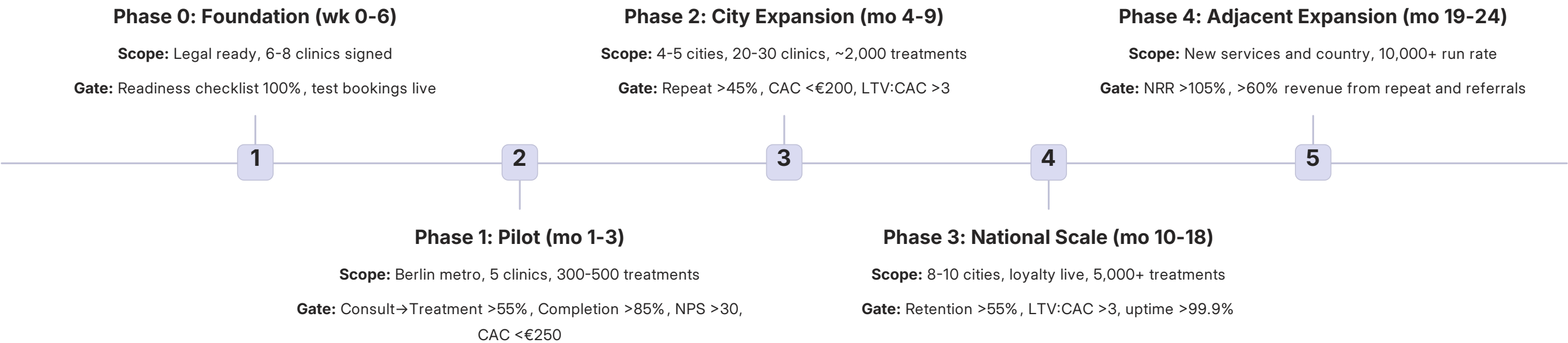


Time to Appointment

Fast scheduling reduces drop-offs and improves patient experience.

NSM = paid treatments per month | Pilot math at NSM 500, 50% repeat, S2 CAC €135 | Contribution $500 \times €105 = €52.5k$ | CAC on 250 new = €33.8k | Net after CAC = €18.8k before fixed

Rollout Plan with Milestones, Gates, and Workstreams



R&D (Engineering + Data)

- **P0:** Booking engine, payments, events pipeline
- **P1:** Trust module, price chip, one-click booking, reminders
- **P2:** Routing v2, practitioner dashboard, referrals
- **P3:** Mobile app, churn model, QA photo review
- **P4:** Marketplace API, multi-treatment bundles

Platform & Ops

- **P0:** KYC, payout rails, clinic onboarding SOP
- **P1:** SLA 99.5%, support macros, consent e-docs
- **P2:** Multi-city routing, inventory tie-in, room scheduling
- **P3:** Quality audits, delisting rules, cost per treatment down
- **P4:** Intl tax, localization, compliance pack

Marketing & Growth

- **P0:** Brand basics, landing, tracking, email lists
- **P1:** Existing patient email, IG micro-creators, SEO 10-15 posts
- **P2:** Google Ads, retargeting, referral program
- **P3:** National PR, SEO top 3 in target terms
- **P4:** Cross-border playbook and partners

Partnerships & Clinical

- **P0:** 6-8 clinics signed, malpractice cover, protocols
- **P1:** 10-15 shadowed treatments, QA checklist
- **P2:** 3-city practitioner cohorts, training program
- **P3:** National network SLAs, quarterly audits
- **P4:** New treatment vendors, bulk pricing

NSM = paid treatments per month

Key KPIs



Acquisition & Activation

- Visitor → Consult booking: 5-7%
- CAC: <€200 by Phase 2, <€175 by Phase 3
- **Guardrail:** Payback $\leq$ 1.5 treatments; stop if > 2



Conversion & Funnel

- Consult → Treatment: 60-70%
- No-show rate: <10%
- Time to appointment < 7 days
- **Guardrail:** No-show >15% or consult completion <75% triggers investigation



Retention & Engagement

- 180-day repeat rate: $\geq$ 50%
- 2.0 → 2.3 → 2.6 treatments per year by phase
- **Guardrail:** Annual retention <40% is warning sign



Satisfaction & Referral

- NPS $\geq$ 30 MVP, $\geq$ 35 P2, $\geq$ 40 P3, 50+ long term
- Referral rate: >20% by Phase 3
- Refunds < 3%
- **Guardrail:** NPS <20 or complaint spikes trigger quality review

Key Risks, Guardrails, Mitigations

Regulatory

Risk: Country-by-country telemed rules delay launches.

Guardrail: Market legal sign-off current; SOPs reviewed quarterly.

Mitigation: Local counsel, licensed MDs, templated e-consent, ad-compliance checklist.

Clinical & Safety

Risk: Adverse outcomes or protocol drift.

Guardrail: Adverse events = 0 reportable; refund rate $\leq$ 3%.

Mitigation: Vet clinics, standard protocols, malpractice cover, 24/7 medical line.

Partner Reliability

Risk: Clinic staffing or inventory gaps cause cancellations.

Guardrail: No-show rate $\leq$ 10%; time-to-appointment < 7 days.

Mitigation: Multi-clinic per city, performance clauses, Botox inventory tie-in.

User Adoption & Trust

Risk: Skepticism about injectables via a platform.

Guardrail: NPS $\geq$ 35; complaint spikes trigger QA.

Mitigation: Doctor credentials, real reviews, upfront pricing, fast support.

Financial & Market

Risk: Mispricing or CAC creep compresses margin.

Guardrail: LTV:CAC $\geq$ 3:1; CAC payback < 2 treatments.

Mitigation: Price tests, CAC dashboards, repeat-engine programs, monthly reviews.

Data & Privacy

Risk: GDPR, PSD2 compliance breaches.

Guardrail: No material breaches reported.

Mitigation: DPA agreements, DPO oversight, regular security audits.

Value Creation – Therapists, Clinics, Platform

Therapists

Clinics

Platform (Company)

Front stage:

- | | | |
|---|---|---|
| <ul style="list-style-type: none">• accept app slots• run consult and consent• deliver treatment with photos• one-click rebook + review. | <ul style="list-style-type: none">• branded page with real-time price and slots• instant checkout• on-site checklist. | <ul style="list-style-type: none">• trust-first landing with transparent price• book now or book consult• smart reminders and chat. |
|---|---|---|

Back stage:

- | | | |
|--|---|--|
| <ul style="list-style-type: none">• sync availability with PMS• follow SOPs and e-docs• auto-invoice with T+2 payout• maintain skills tags. | <ul style="list-style-type: none">• inventory tie-in per booking• staff and room rostering• compliance pack storage• automated payouts and reconciliation. | <ul style="list-style-type: none">• routing engine (patient → clinic → therapist)• payments and KYC• data pipeline for tests and cohort alerts• quality loop with NPS and photo QA. |
|--|---|--|

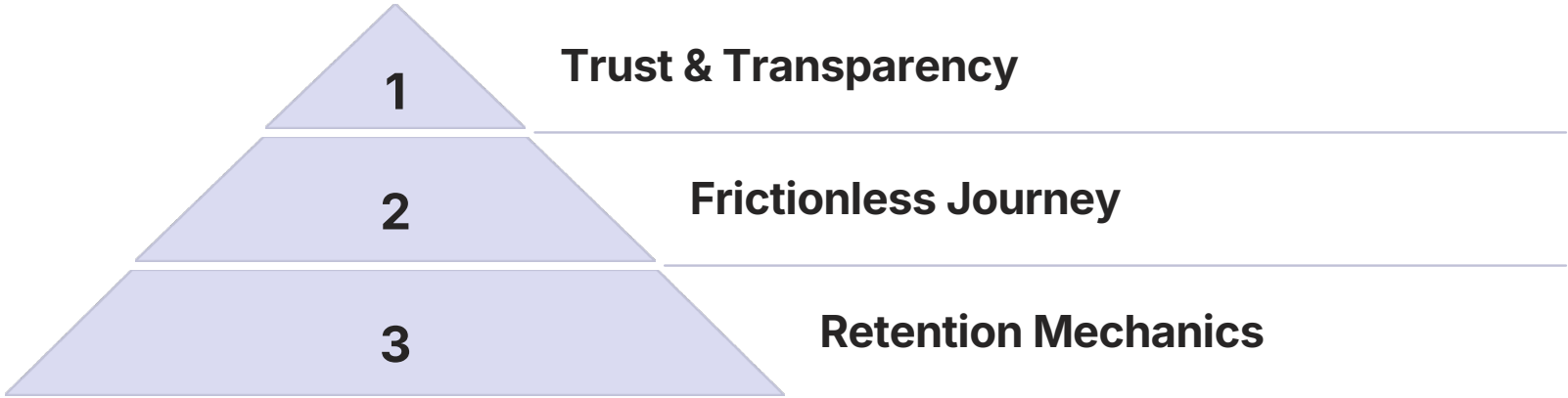
Value:

- | | | |
|---|---|--|
| <ul style="list-style-type: none">• higher paid-hour utilization• fewer no-shows• faster payouts with less paperwork. | <ul style="list-style-type: none">• predictable demand that fills gaps• lower admin per treatment• margin lift via bulk toxin purchasing. | <p>contribution per treatment = price × take – variable cost
example: €400 × 30% – €15 = €105</p> <ul style="list-style-type: none">• higher LTV from repeat and cross-sell• network effects across cities. |
|---|---|--|

Metrics:

- | | | |
|--|---|---|
| <ul style="list-style-type: none">• utilization• no-show rate• NPS• repeat-patient share. | <ul style="list-style-type: none">• calendar occupancy• on-time start rate• margin per treatment• refund rate. | <ul style="list-style-type: none">• NSM paid treatments per month• consult → treatment conversion• 180-day repeat• CAC• LTV• LTV:CAC• payout latency. |
|--|---|---|

Path to Success + Pilot Ask



Trust: credentials, reviews, clear price • Journey: consult → one-click booking • Retention: reminders, membership, plan

Next gate: Hit targets and guardrails for 2 consecutive months, expand to Munich and add loyalty pricing.

Decision Today

- Approve Berlin pilot with 5 clinics
- Budget cap €75k for CAC + build
- Legal green light for e-consent, PSD2 payments, DPA

Pilot Targets M1–M3

- NSM: ≥ 300 paid treatments by month 3
- Consult → treatment: $\geq 60\%$
- Repeat at 180 days: $\geq 50\%$
- CAC (S2 Switchers): $\leq \text{€}150$
- Contribution per treatment: $\text{€}105$

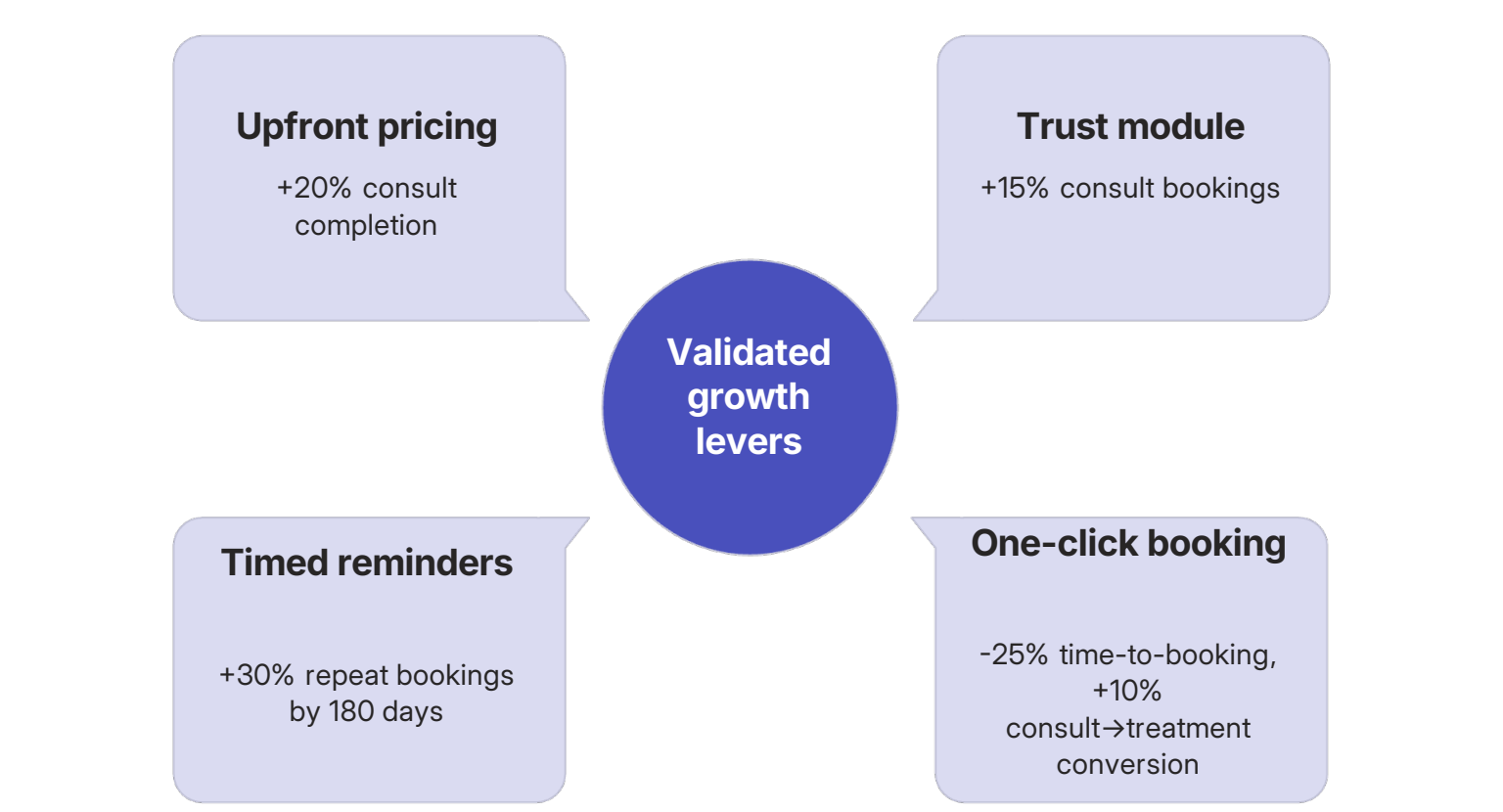
Guardrails

- NPS: ≥ 35
- No-show: $\leq 10\%$
- Refunds: $\leq 3\%$
- Adverse events: 0 reportable

Product-Led Growth Testing Framework

Hypothesis	Test Design	Primary metric	Success Criteria	Guardrails
Trust Gap Prevents Conversion	A/B landing: doctor credentials + video testimonials vs minimal info	ConsultBooked rate, NPS post-consult	≥ +15% consult bookings	No-show ≤ 10%, refunds ≤ 3%, NPS ≥ 35
Booking Friction Causes Drop-Off	A/B at end of consult: BookNow one-click vs "We will email you"	Consult→Treatment conversion, time-to-booking	≥ +10% conversion and -25% time-to-booking	Same
Lack of Follow-Up Lowers Retention	Cohort test: SMS/email at 90/105/120 days to 50% vs control	RepeatBooked by day 180	≥ +30% repeat rate	Same
Price Opacity Deters First-Timers	A/B: "Botox from €350" with breakdown vs "Contact for pricing"	ConsultCompleted rate	≥ +20% consult completion	Same

Instrumentation note: Track LPViewed, PriceChipClick, AreaSelect, ClinicSelect, SlotView, BookNowClick for attribution and funnel diagnosis.



Next 30-Day Action Plan

Key Decisions (ASAP)

- 1. Pilot city/market selection (Berlin confirmed?)
- 2. Finalize pricing model and offering details
- 3. Lock in revenue share structure (30/70 split validation)
- 4. Tech development vs. purchase decision
- 5. Approve legal/risk framework

01

Team & Resources

Form core project team: PM, operations lead, medical advisor. Weekly check-ins. Identify hiring needs.

02

Partner Recruitment

Outreach to pilot city clinics. Goal: 3 clinics provisionally signed. Line up telemedicine doctors.

03

Product Build

Complete landing page and consultation booking flow. Integrate scheduling and video platform. Working prototype for internal testing.

04

Analytics & Tracking

Implement event tracking for funnel. Set up basic KPI dashboard for launch monitoring.

05

Marketing Prep

Create SEO content, draft announcement email, prepare ad campaigns, engage local influencers for soft launch.

06

Compliance & Training

Finalize patient forms, legal review, train telemedicine doctors on workflow, brief customer support team.

07

Launch Readiness Check

Internal end-to-end dry run by Day 30. Test entire patient journey, iron out glitches, prepare for real users.

Thank You!

We appreciate your valuable time and attention today as we presented the DoktorABC Aesthetics MVP launch plan.

now for Q&A

& a sneaky demo on Mobile & Web.

Citation Index:

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Problem Definition	3	Patients face trust gaps, booking friction, slow follow-up; response time strongly affects conversion	2.3% visitor→inquiry, 66%+ inquiry→patient, 10x uplift when responding <5 minutes	[10],[11],[12],[13]	4
				[14],[15],[16],[17],[18],[19],[20],[21],[22],[23]	
Goals & Metrics (PLG)	6–7,14	Use a North Star + input metrics; PLG-style KPI stack for growth and retention	North Star concept, LTV:CAC≥3:1 benchmark, PMF and PLG metric patterns		10
Conversion Benchmarks	6–7	Set realistic funnel targets for visit→consult and consult→treatment based on industry data	~2–3% visit→inquiry; ~60–70% inquiry→patient for clinics; strong impact of fast follow-up	[10],[24],[25],[26],[27]	7
Product Levers	8–11	Landing, booking, reminders, memberships, and galleries as main growth & retention levers	One-click booking, 90–120 day reminders, before/after galleries, referral programs	[28],[29],[30],[31],[32],[33],[34],[35]	8
Unit Economics	11	Validate that LTV and LTV:CAC are viable; show importance of repeat for profitability	Healthcare LTV:CAC>=3:1; repeat clients drive ~80% of revenue; CAC bands for healthcare	[36],[37],[38],[39],[40],[41]	6
Persona Demographics	4–5	Demographic basis for segments: First-timers, Switchers, Upgraders; age and gender split	~94% female; 57% Botox patients 40–54; 24% under 34 and growing	[42],[43],[44],[45],[46],[47]	6
Landing Page Design	9	Evidence for trust-heavy landing, transparent pricing, healthcare UX and PLG patterns	Trust signals, medical UX best practices, long-form landing content, telehealth D2C patterns	[48],[49],[50],[51],[52],[53],[54],[55],[56],[57],[58],[59],[60]	13
S2 Switcher Psychology	4–5	Why Switchers churn from current clinics and are open to a better funnel	Switching driven by dissatisfaction, variety seeking, negative WOM	[61]	1
S3 Long-Term Market	4–5,backup	Fence-sitters and Upgraders; why long-term market can double in 5 years	15–20% intend to try injectables in 12 months; market expected to roughly double	[62],[63]	2
North Star & PLG Narrative	6–7,14	Definition of NSM and supporting metrics; PLG-style growth model	NSM as ‘paid treatments per month’; supporting metrics: consult→treatment, 180-day repeat, CAC, LTV	[14],[18],[20],[21],[22],[23],[36],[41]	8